



Assessing Patient Capacity SOP

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	All clinical areas	All staff, Medical, Nursing, Allied Health

Purpose

This standard operational procedure (SOP) aims to assist clinicians in their assessment of a patient's capacity to consent to or refuse medical treatment.

Links within document

- [Appendix I: Patient Capacity Assessment Form](#)

Definitions

Capacity	In the context of medical treatment, a patient has capacity if they are capable of understanding the nature, purpose and consequences of the proposed treatment or not to have the proposed treatment. Capacity must always be assessed in the context of the decision that is made. All adults are presumed to have capacity unless there is evidence to suggest otherwise.
Consent	In the context of health care, consent to treatment is a person's agreement that a health professional can proceed to perform a specific treatment/procedure etc.

The law recognises the rights of a competent individual to make decisions about their medical care and this concept is inherent in principle of an individual's autonomy. Consent to treatment will be invalid if it has been given by a person who lacks decision making capacity. To act against an individual's wishes who is competent, has capacity, may open a clinician to medico-legal issues.

Consideration is to be given to mature minors, i.e. where a consumer is assessed as having sufficient intellectual and emotional maturity and competence to understand information relevant to a proposed treatment, including its risks, benefits and alternatives, then they can consent to or decline that treatment on their own behalf.

Refer to [Department of Health Consent to Treatment Policy](#).

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Procedure

Key considerations when assessing capacity

- A patient's capacity is their mental ability to make decisions for themselves
- An adult patient has capacity to make decisions unless proven otherwise
- Capacity assessment is specific to the decision being made, and a person may have capacity to make some types of decisions, but not others
- Ability to make decisions varies from person to person, from situation to situation and is influenced by the person's mental and/or physical state
- A patient's capacity may fluctuate or change over time and can be affected by environmental and other factors (e.g., medications, stress, anxiety, depression, psychosis, drugs and alcohol)
- If a patient is deemed to have capacity, they are entitled to make decisions which others may not agree with or deem to be unwise
- Ensure the patient's decision is made voluntarily without coercion from others
- The choice which a patient makes will be based on the amount and quality of information provided to them in a manner they can understand. The patient needs to be given sufficient time to think about their decision after having been given the relevant information
- No assessment is based on age, appearance, condition, diagnosis and behaviour
- Consider capacity assessment for minors/mature minors, as required

When should capacity be formally assessed?

- The person is making decisions now which puts the person or others at significant risk of harm
- The person is making decisions that are different from their usual decisions or conflicts with their usual preference
- A previous assessment has highlighted the person lacking decision making capacity in the past
- If the person is experiencing noticeable memory problems, or is disorientated to time, place and person

Who is responsible to assess capacity?

- Assessing a patient's capacity to consent to medical treatment remains the responsibility of the medical team proposing the treatment
- Referral to Royal Perth Hospital (RPH) Consultation-Liaison (C-L) Psychiatry Service or the Bentley Health Service (BHS) Assessment and Treatment Team (ATT) for input into evaluation of a patient's capacity to consent is appropriate when:
 - There is a question of an active mental illness (e.g., depression or psychosis) that is impacting on decision-making capacity
 - There is uncertainty about underlying cognitive difficulties that may be impacting on decision-making capacity
 - In complex cases for a second opinion

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Procedure - Who is responsible to assess capacity? cont'd

- A psychiatric assessment can be requested via eReferral to the following teams.

Royal Perth Hospital	
In Hours	RPH C-L Team
After Hours/Weekends and Public Holidays	Duty Psychiatric Registrar
Bentley Health Service	
In Hours (0800 – 2200)	ATT
After Hours/Weekends and Public Holidays	Psychiatry Duty Medical Officer

Prior to assessing capacity

Prior to assessing the patient's capacity, the assessing clinician should have an understanding of:

- The diagnosis and nature of the patient's condition that has led to the offer of treatment for them
- The likely results, risks and benefits associated with the proposed treatment and the benefits that the patient can reasonably expect. The assessing clinician should be able to discuss with the patient the consequence of alternative treatments and no treatment
- Obtain information from the patients' family or significant others (ensuring confidentiality) to gain understanding of the patients' values during decision making.

How to assess capacity?

The process to assess the patient's capacity to make medical decision must include the following components:

1. Understanding the information

- Does the patient have a factual understanding of the information presented to them to make a decision regarding the treatment offered?
 - Does the patient understand their role as the decision maker?
 - The patient should understand in broad terms the nature, the purpose, the risks and benefits of the intervention, but also the risks of not carrying out the intervention and the risks and benefits of alternative interventions.

4 Elements to Assess Capacity

- Understanding
- Retaining
- Appreciation
- Communication of choice

2. Retaining the information

- Does the patient remember the relevant information required for them to make a decision?

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Procedure - How to assess capacity? cont'd

3. Appreciation

- Does the patient have an appreciation for the consequences for them in making their decision?
 - **Communication** The patient should demonstrate an awareness of their medical situation
 - In discussion with the clinician the patient should exhibit comparative and consequential reasoning as they weigh up the nature and probability of material risks involved in accepting treatment; or refusing it
 - The patient should demonstrate the ability to weigh the risks and benefits of a proposed treatment through a logical process. Their decision may not be as the clinician recommends but it has a consistency with the values and benefits of the patient

4. Communication of choice

- The patient has manipulated the information through items 1-3 and expressed a choice through a logical process. The focus of the capacity assessment is on the rationality of the patient process in making a decision and not on their decision itself
- Inability to express a choice leads to a presumption of incompetence unless the patient is hard of hearing or has absence of speech due to health condition or illness. A consistently impaired patient (dementia, cerebral vascular accident [CVA]) may still have some capacity to express their wishes
- The expression of the patient's choice is consistent over time

What are strategies to improve decision-making capacity?

Ensure information regarding the treatment has been clearly communicated using method of communication that will be understood including audio-visual aids, leaflets, interpreter services or information in another language. Involving the family/carer can also assist.

Process to document capacity

The assessment of capacity needs to clearly be documented into the patient medical record with the following requirements:

- Reason why an assessment was undertaken
- Reasoning in accordance with each of the **four elements to assess capacity**
- Date and time of assessment
- Name and signature of clinician who undertook the assessment

Refer to Appendix I the *Patient Capacity Assessment Form* (sample) which can be completed. Alternatively, address the above points in the patient integrated notes.

Note the *Patient Capacity Assessment Form* can be ordered.

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Substitute decision makers

- If a patient is assessed as having a lack of decision-making capacity, a discussion between the family and carers for the patient and the clinician should occur as a first approach
- It is recognised a patient's capacity is dynamic in nature and may change over time requiring a further assessment at a later stage in accordance with the process outlined above
- If the treatment can be delayed until capacity has improved, that is preferred (e.g., treatment of depression, resolution of delirium)
- If the treatment or intervention is considered more urgent and a person has no capacity to consent the *Guardianship and Administration Act 1990* outlines a hierarchy of substitute decision makers for adults that are lacking in decision-making capacity. For further information refer to the [Office of the Public Advocate](#)
- If an Advanced Health Directive does not exist or does not cover the treatment decision required health professionals must obtain a decision for non-urgent treatment from the first person in the hierarchy who is aged 18 years and older, and has full legal capacity, and is willing and available to make a decision

Note: The clinician in making the recommendation and plan for action needs to ensure the decision is the least restrictive option for the patient's rights and freedoms.

Facilitator

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Assessing Patient Capacity SOP through consultation with Aboriginal Health Strategy team, RPBG Aboriginal Health Liaison Officer Coordinator and Team, Wungen Kartup, Aboriginal Acute Care Coordination Team and Manager Aboriginal Health to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 803).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Related policies, practice standards, clinical guidelines

[Department of Health Policy Hub](#)

- [WA Health Consent to Treatment Policy](#)

Other

- Mental Health Act 2014
- Mental Health Commission
- A Capability Framework for working with people with intellectual disability and co-occurring mental health issues
- [Office of the Public Advocate Making Treatment Decisions](#)

Related forms

- Patient Capacity Assessment Form
- Consent to Treatment Form

Related National Standards

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 1: Clinical Governance

Standard 5: Comprehensive Care

Standard 6: Communicating for Safety

Standard 8: Recognising and Responding to Acute Deterioration

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Appendix I: Patient capacity assessment form

UMRN:
Surname:
First Name:

Name of clinician undertaking assessment:	
Position title:	
Date/Time:	
Signature:	

Why is the patient needing to be assessed for capacity? (i.e. type of treatment, requesting to be discharged?)

Assessment of Capacity

1. Understanding

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Appendix I: Patient capacity assessment form cont'd

2. Retaining
3. Appreciation
4. Communication of choice
Other comments or considerations
Recommendation and Action taken

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